INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Carboplatin AUC 4 and fluorouracil with radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00591
Version 4

> Day 1: carboplatin; Days 1–4: 5-FU. Every 21 days ×3, during radiotherapy (RT).

Treatment stages

01 **Concurrent chemoradiotherapy**
Day 1: carboplatin; Days 1–4: 5-FU. Every 21 days ×3, during RT.

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Carboplatin AUC 4 and fluorouracil with radiotherapy
National Cancer Control Programme (NCCP) 00591

Head and neck: chemoradiotherapy

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

What each medicine does

Chemotherapy is given with radiation to improve control of cancer in the head and neck. Your team confirms the treatment aim and why carboplatin is suitable for your circumstances.

PLATINUM CHEMOTHERAPY

Carboplatin

Carboplatin



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Carboplatin interferes with DNA repair and cell division. Its dose is calculated from kidney function. Blood counts can fall, including platelets that help stop bleeding. It is a different prescription from cisplatin. AUC is a prescribing target for drug exposure, not the number of milligrams in your bag; your clinical team calculates the dose using kidney function.

DNA damage → Cell division disrupted → Cancer growth reduced

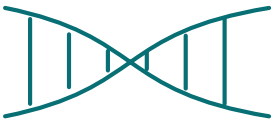
What to watch for

Report fever, unusual bruising or bleeding, marked tiredness or an infusion reaction.

ANTIMETABOLITE CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts production of DNA and RNA

Fluorouracil, often shortened to 5-FU, interferes with the building blocks used to make DNA and RNA, which cells need to grow. It runs through a vein over the prescribed hours. Dihydropyrimidine dehydrogenase (DPD) is an enzyme that breaks it down. Testing for DPD deficiency before treatment reduces risk but cannot rule out every severe reaction.

Cell building blocks disrupted → Cells cannot divide normally → Pump provides prescribed exposure

What to watch for

Report severe diarrhoea, mouth sores, chest pain, sudden confusion, sore palms or soles, or unexpectedly early severe symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Understanding the evidence

The national regimen brings together research and specialist guidance for a specific treatment setting.

The GORTEC 94-01 evidence cited by NCCP concerns four-day carboplatin 70 mg/m² plus fluorouracil with radiation in advanced oropharyngeal cancer. Its numerical outcomes should not be treated as an exact estimate for every head and neck site or for the separate carboplatin AUC 4 schedule.

CHAPTER 02

Your schedule, tests and appointments

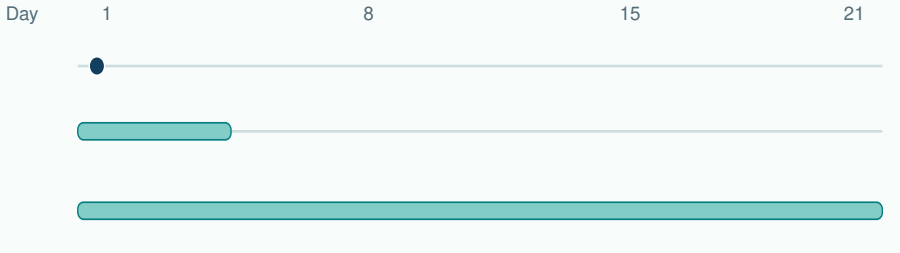
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment stages

Carboplatin AUC 4 and fluorouracil with radiotherapy

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAY 1: CARBOPLATIN; DAYS 1–4: 5-FU. EVERY 21 DAYS ×3, DURING RADIOTHERAPY (RT).

Concurrent chemoradiotherapy

Up to 3 cycles

Carboplatin AUC 4 on Day 1, with fluorouracil 600 mg/m²/day on Days 1–4, every 21 days for 3 cycles, only concurrently with radiotherapy.

Chemotherapy is given only during radiotherapy. If radiation is interrupted, contact the team before the next chemotherapy dose. Radiation oncology sets the radiation visits and follow-up.

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Routine safety tests, symptom-driven tests and response scans have different purposes.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00591 v4: retrieved official regimen

Tests in your plan

Full blood count

WHEN

Before each cycle.

Checks infection-fighting cells, oxygen-carrying cells and platelets for clotting.

Kidney and liver tests

WHEN

Before each cycle: full blood count, kidney and liver tests. Review weight, intake, swallowing, mouth symptoms, hands and feet. Measured kidney function is preferred when feasible; audiology is arranged when indicated.

Checks treatment handling and organ safety.

DPD assessment

WHEN

Before first fluorouracil unless previously tested.

Checks risk of unusually severe fluorouracil toxicity; a normal result does not exclude all risk.

Hearing assessment

If part of your plan

WHEN

When clinically indicated.

Assesses existing or new hearing changes and tinnitus.

THE WHOLE TREATMENT JOURNEY

Carboplatin AUC 4 and fluorouracil with radiotherapy: treatment and checks

Locally advanced head and neck squamous cell cancer when cisplatin-based treatment is unsuitable.

Carboplatin AUC 4 on Day 1, with fluorouracil 600 mg/m²/day on Days 1–4, every 21 days for 3 cycles, only concurrently with radiotherapy.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Next cycle / phase	After treatment
Blood and organ checks NCCP	Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. DPD testing before first fluorouracil unless already tested; kidney measurement and hearing assessment as indicated. Arrange dental and swallowing/ nutrition review before head and neck radiation.	Before each cycle: full blood count, kidney and liver tests. Review weight, intake, swallowing, mouth symptoms, hands and feet. Measured kidney function is preferred when feasible; audiology is arranged when indicated.	Extra tests may be needed for symptoms or abnormal results.	Review results, recovery and earlier toxicity before another dose.	The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.
Treatment and appointments NCCP	Confirm the complete treatment plan.	Carboplatin is given over 30 minutes on Day 1; its dose is calculated from kidney function. Fluorouracil runs continuously over 96 hours; the source also permits separate 22-hour infusions on Days 1–4. Confirm the actual pump or bag-change and disconnection times.	Follow written medicine and contact instructions.	Chemotherapy is given only during radiotherapy. If radiation is interrupted, contact the team before the next chemotherapy dose. Radiation oncology sets the radiation visits and follow-up.	Record completion and the named follow-up team.

Care / check	Before starting	Treatment visits	Between visits	Next cycle / phase	After treatment
Mouth, weight and swallowing NCCP	Assess symptoms, teeth, eating and drinking.	Review weight, intake and mouth symptoms.	Call for difficulty swallowing, worsening pain or weight loss.	Dietitian and swallowing support according to need.	Continue rehabilitation and dental care if advised.

What each check is for

1 **Treatment course**

Day 1: carboplatin; Days 1–4: 5-FU. Every 21 days ×3, during RT.

Chemotherapy is given with radiation to improve control of cancer in the head and neck. Your team confirms the treatment aim and why carboplatin is suitable for your circumstances.

Ask / act: Confirm active medicines, safety checks and support plan.

2

Review / next phase

After the planned course and recovery review

Review response and recovery.

Ask / act: Confirm actual dates and the next prescription.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · undefined

undefined

undefined

2 · undefined

undefined

undefined

3 · undefined

undefined

undefined

4 · undefined

undefined

undefined

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A peripherally inserted central catheter (PICC) remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before your first visit

Full blood count, kidney and liver tests; review medicines, allergies, weight, oral intake and symptoms. DPD testing before first fluorouracil unless already tested; kidney measurement and hearing assessment as indicated. Arrange dental and swallowing/nutrition review before head and neck radiation.

STEP 2

Time in the unit

Carboplatin is given over 30 minutes on Day 1; its dose is calculated from kidney function. Fluorouracil runs continuously over 96 hours; the source also permits separate 22-hour infusions on Days 1–4. Confirm the actual pump or bag-change and disconnection times.

STEP 4

Leaving the unit

Confirm the next tests, medicines, treatment dates and verified daytime and out-of-hours contact numbers.

STEP 5

Practical help

Ask about transport, work, costs, communication needs and whether a support person can attend.

Your exact treatment schedule

Tests and preparation

During treatment

Before each cycle: full blood count, kidney and liver tests. Review weight, intake, swallowing, mouth symptoms, hands and feet. Measured kidney function is preferred when feasible; audiology is arranged when indicated.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Carboplatin	 **Day 1** AUC 4 on Day 1 only; kidney-based dosing.
Fluorouracil infusion	 **Days 1–4** 600 mg/m²/day, continuous 96-hour infusion or the explicitly prescribed daily 22-hour alternative.
Radiotherapy course: separate visit calendar	 **Days 1–21** Separate radiation calendar; chemotherapy only while concurrent radiotherapy is confirmed.

Prescribed timing: Carboplatin AUC 4 on Day 1, with fluorouracil 600 mg/m²/day on Days 1–4, every 21 days for 3 cycles, only concurrently with radiotherapy.

Review and next step: Chemotherapy is given only during radiotherapy. If radiation is interrupted, contact the team before the next chemotherapy dose. Radiation oncology sets the radiation visits and follow-up.

Safety throughout: Symptoms and urgent-contact advice apply throughout treatment and after the last dose.

Why these bands stay broad: This is a reference cycle, not a personal appointment calendar. Side effects can occur on any day; the recovery interval is not a guarantee of safety. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00591 v4: retrieved official regimen ↗

There are no guaranteed "safe days". Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Between-treatment notes

Day 1–4: Days 1–4: chemotherapy and radiation

Carboplatin is given over 30 minutes on Day 1; its dose is calculated from kidney function. Fluorouracil runs continuously over 96 hours; the source also permits separate 22-hour infusions on Days 1–4. Confirm the actual pump or bag-change and disconnection times. Confirm every infusion, pump/bag-change and radiation appointment.

Day 5–21: Ongoing radiation and recovery

Radiation follows its separate calendar. Mouth soreness and swallowing problems can build up and continue after treatment. Tell the team early about pain, weight loss or inability to drink.

Support around the medicines

1 Preventing and treating sickness

Use the prescribed medicines for the active treatment day. The regimen is classified as high sickness risk.

2 Fluorouracil and your infusion

DPD testing checks for an enzyme deficiency that can increase toxicity. Normal testing does not eliminate risk. Call urgently for chest symptoms or unusually early severe diarrhoea, mouth soreness or confusion.

3 Mouth care and nutrition

Follow your unit's oral-care plan. Tell the team early about mouth pain, weight loss, choking or difficulty eating and drinking. Dietetic and swallowing support can help.

4 Medicine review

Bring prescription, non-prescription and herbal medicines. Check interactions, allergies and any changes with the oncology pharmacist.

5 Recovery and practical support

Ask about fatigue, work, transport, caring responsibilities, emotional support and help with appointments.

Understanding your diagnosis

Tumour tests and safety tests

Your team explains cancer-specific tests, including Epstein–Barr virus tests where relevant in nasopharyngeal cancer. These are distinct from blood tests used for medicine safety. No universal tumour-marker schedule is prescribed here.

Your treatment aim and choices

Deciding together

Discuss expected benefit, possible lasting effects, suitable alternatives and the option of declining treatment. Tell the team what matters most to you.

Symptoms and when to call

Between visits

Keep a record of symptoms and medicines taken. Do not wait for a routine appointment when symptoms are worsening.

Mouth, eating and swallowing

Speak up early

Tell the team about pain, dry mouth, taste changes, choking, reduced intake or weight loss. Ask for a personalised oral-care, dietetic and swallowing plan.

Radiation-related care

If you are having head and neck radiation, follow the radiation team's dental, skin, saliva and swallowing advice during and after treatment. Radiation appointments and late-effect checks have their own plan.

Living with treatment

Activity and rest

Pace activity according to symptoms. Ask for help with fatigue, sleep, work, travel or caring responsibilities.

Sexual health and fertility

Discuss fertility preservation before treatment where appropriate. Ask about contraception, pregnancy, breastfeeding and medicine-specific timeframes.

Infusions and access

Your infusion arrangement

Line problems

Report pain, redness, swelling, leaking or a wet dressing. Do not adjust infusion equipment yourself; contact the unit.

After this course

Recovery and follow-up

The team follows recovery, response and persistent side effects. Scans and tumour tests follow the individual plan; there is no universal scan or tumour-marker schedule in this regimen.

Questions for your team

My plan

What is the treatment aim? Which tests are mandatory? What happens if a treatment day is missed?

My support

Which medicines do I take and when do I stop them? Whom do I contact overnight?

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

Try explaining the plan back: Can you explain your treatment days, required blood tests, support medicines and whom to call if unwell? Ask the team to write down any unclear part.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Chemotherapy visit

Active medicines: Carboplatin + Fluorouracil

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Ondansetron	16 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	12 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Day 1 Clinician/pharmacy draft; reconcile to the individual prescription.
Aprepitant	80 mg PO OD	Days 2-3 Clinician/pharmacy draft; reconcile to the individual prescription.
Dexamethasone	8 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.
Metoclopramide	10 mg PO TDS	Subsequent days / as directed Clinician/pharmacy draft; reconcile to the individual prescription.
Olanzapine	5 mg PO OD	Days 2-4 Clinician/pharmacy draft; reconcile to the individual prescription.

Additional phase-specific support

- Carboplatin is given over 30 minutes on Day 1; its dose is calculated from kidney function. Fluorouracil runs continuously over 96 hours; the source also permits separate 22-hour infusions on Days 1–4. Confirm the actual pump or bag-change and disconnection times.
- Before each cycle: full blood count, kidney and liver tests. Review weight, intake, swallowing, mouth symptoms, hands and feet. Measured kidney function is preferred when feasible; audiology is arranged when indicated.
- Chemotherapy is given only during radiotherapy. If radiation is interrupted, contact the team before the next chemotherapy dose. Radiation oncology sets the radiation visits and follow-up.
- The local CUH source is retained as a reference list, not as the national antiemetic standard. Nystatin/chlorhexidine are displayed as local mouth-care adjuncts, not universal antiemetic or mucositis prophylaxis. Verify current local oral-care policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

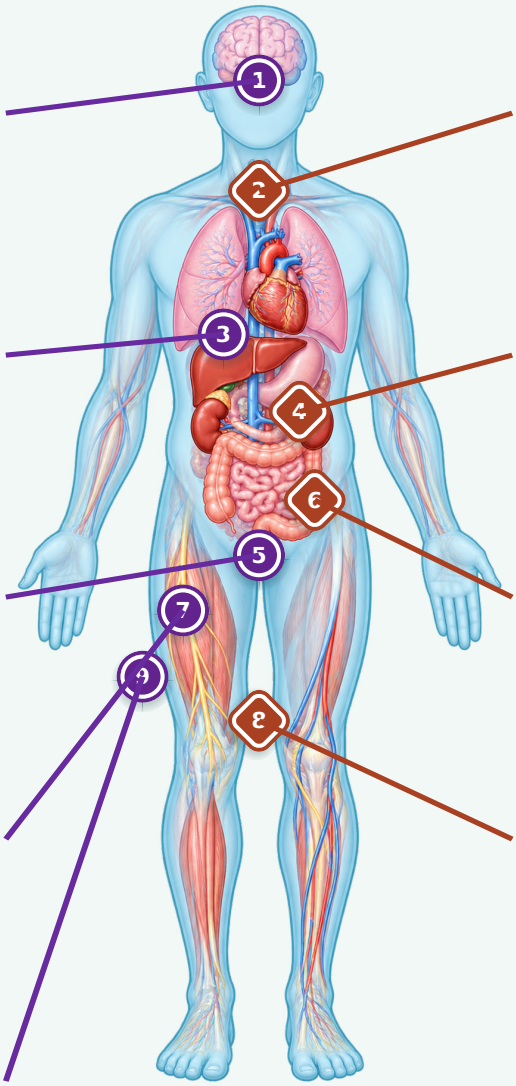
1 Fatigue and concentration
Carboplatin + Fluorouracil
Tiredness can affect activity and concentration. Balance activity and rest; report sudden confusion.

3 Breathing
Carboplatin + Fluorouracil
New or worsening breathlessness, cough or chest symptoms need urgent advice; lung inflammation can be serious.

5 Sickness and bowels
Carboplatin + Fluorouracil
Take prescribed anti-sickness medicines. Report persistent vomiting, diarrhoea, dehydration or inability to drink.

7 Skin and infusion reactions
Carboplatin + Fluorouracil
Report severe rash, blistering, facial swelling or breathing symptoms promptly. Tell staff about pain at the infusion site.

9 Hands and feet / chest
Fluorouracil
Report sore palms or soles, chest pain or unusually early severe symptoms, even after normal DPD testing.



2 Mouth and swallowing
Carboplatin + Fluorouracil / radiotherapy
Report soreness, ulcers or difficulty swallowing. Ask early about mouth care, pain relief and nutrition.

4 Blood and infection
Carboplatin + Fluorouracil
Fever, shivering, sudden illness, unexplained bruising or bleeding needs urgent oncology advice.

6 Kidneys and fluid balance
Carboplatin + Fluorouracil
Report reduced urine, swelling or poor fluid intake. Blood tests help detect kidney injury.

8 Fertility and pregnancy
Carboplatin + Fluorouracil
Discuss fertility preservation, contraception, pregnancy and breastfeeding before treatment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivering, sudden illness, unusual bleeding, new breathlessness, persistent vomiting or inability to drink needs urgent oncology advice. Severe chest pain, severe breathing difficulty, collapse or a seizure is an emergency: call 112/999.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Record actual treatment dates and dose changes. Your clinician confirms the course length and any later phase.

· copy this sheet for another cycle of the same phase.

Carboplatin · Platinum chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Fever / chills	□ No fever or chills	□ New chills / feeling unwell: call	□ Fever: use the written urgent plan now	□ Fever with marked weakness: urgent assessment	□ Collapse / confusion / breathing difficulty: 112/999
Bleeding / bruising	□ None	□ New small bruise / brief bleed: report	□ Repeated bleeding: call today	□ Bleeding that will not stop: urgent help	□ Heavy bleeding / collapse: 112/999
Infusion symptoms	□ None	□ New flushing, chills or itching: tell the nurse immediately	□ Symptoms during infusion: tell the nurse immediately	□ Wheeze, swelling or faintness: immediate urgent help	□ Severe breathing difficulty or collapse: emergency help
Numbness / tingling	□ None	□ Mild change; usual tasks unaffected	□ Limits buttons, writing or walking: call	□ Washing, dressing or walking difficult: urgent review	□ Sudden major weakness / paralysis: emergency help

Fluorouracil · Antimetabolite chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Hand-foot soreness	□ None	□ Redness or tingling without pain	□ Pain limits usual tasks: call	□ Pain limits washing / dressing: urgent review	□ Acute severe illness: emergency help
Heart symptoms	□ None	□ New palpitations: contact team promptly	□ Palpitations with dizziness: urgent assessment	□ Chest pain or fainting: emergency assessment	□ Severe chest pain, collapse or breathing difficulty: 112/999
Fever / chills	□ No fever or chills	□ New chills / feeling unwell: call	□ Fever: use the written urgent plan now	□ Fever with marked weakness: urgent assessment	□ Collapse / confusion / breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Head and neck: chemoradiotherapy · NCCP 00591 v4 · Document revised 11 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Carboplatin AUC4 + 5-FU / RT

Day 1: carboplatin; Days 1–4: 5-FU. Every 21 days ×3, during RT.

Phase: _____ Planned cycles: 3

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Team stamps after each completed cycle.

C1	C2	C3
Date / initials	Date / initials	Date / initials

My treatment information

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.49 · NCCP 00591 v4 · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00591 v4: retrieved official regimen**
   https://healthservice.hse.ie/documents/6988/591_CARBOplatin_AUC4_and_5-Fluorouracil_600.pdf
2. **eviQ: port**
   https://www.eviq.org.au/p/507
3. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
4. **eviQ: pump**
   https://www.eviq.org.au/p/502
5. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
6. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
7. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
8. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
9. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf